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Alana Reyes, MD
Orthopaedic Spine

June 19, 2026

Atty. Elijah Thorne
LSH Law Group
500 Commerce St, Suite 1200
Riverton

Re:	John Doe · DOB 08/14/1991 · MRN LKO-218840
Date of injury:	02/06/2026 (motor vehicle collision)
Your file:	LSH-2026-MVA-770412

Dear Mr. Thorne:

At your request, and with Mr. Doe's written authorization, I am providing my opinion on the cause of the cervical injury for which I have treated him. I saw Mr. Doe on March 5, March 27 and June 12, 2026.

Records reviewed. In addition to my own records and the MRI images, I reviewed:

- Riverton Fire EMS run report RF-26-0206-114, February 6, 2026;
- Riverton Urgent Care visit note, February 20, 2026 (Nina Patel, PA-C);
- Riverton Spine & Rehab initial evaluation, daily notes and discharge summary, February 24 – June 5, 2026;
- Summit Imaging Center cervical MRI report, March 13, 2026 (Helen Park, MD);
- Crescent Pain & Spine procedure note, May 15, 2026 (Omar Haddad, MD);
- Birch Avenue Chiropractic records, May 2021 (Lena Ford, DC).

Summary. Mr. Doe, then 34, was the restrained front-seat passenger in a vehicle struck on his side on February 6, 2026. Paramedics documented neck stiffness at the scene. His neck pain worsened over the following days, numbness in the right thumb and index finger began on February 18, and he was first treated on February 20, 2026. Examination showed a right C6 radiculopathy, and the MRI of March 13, 2026 showed a C5-C6 right paracentral disc herniation contacting the right C6 nerve root, with no significant degenerative change at the adjacent levels. He improved with chiropractic care and therapy and an epidural steroid injection, and reached maximum medical improvement on June 12, 2026 with residual intermittent right-arm paresthesia.

Opinions. Each of the following is held to a reasonable degree of medical probability:

1. **The delayed presentation is clinically consistent with this injury.** A traumatic disc injury commonly presents first as neck pain and stiffness; radicular symptoms evolve over days to weeks as the herniated material and the surrounding inflammation affect the nerve root. Neck symptoms documented on the day of the collision, followed by arm numbness about twelve days later, is the expected course, not an inconsistent one. The two weeks between the collision and his first visit do not suggest a different cause.
2. **The C5-C6 herniation was more likely than not caused by the February 6, 2026 collision.** He had no neck complaints or treatment before the collision; his symptoms began with it; the imaging shows a focal herniation with annular high signal at the symptomatic level and no significant degenerative change at the adjacent levels, a pattern more consistent with an acute injury than with age-related disease in a 34-year-old; and his examination findings match the level and side of the herniation.
3. **The 2021 chiropractic care is unrelated.** The Birch Avenue Chiropractic records describe three visits in May 2021 for low-back stiffness after moving furniture, lumbar findings only, a normal cervical examination, and resolution with release to full activity. They show no prior neck complaint and have no bearing on this cervical injury.

4. **The treatment was reasonable and necessary** for the injury described, and the future care outlined in my June 12, 2026 note (one to two repeat cervical epidural steroid injections within 24 months, at about $3,400 each) is reasonably anticipated.

I reserve the right to amend these opinions if additional records become available.

Sincerely,

Alana Reyes, MD

Alana Reyes, MD
Orthopaedic Spine Surgery, Lakeside Orthopedics